Editor
2026-07-08 07:17 AM

Subject: [JOIV] Editor Decision DELETE

Prof Lailil Muflikhah:

We have reached a decision regarding your submission to JOIV : International Journal on Informatics Visualization, "Attention-Enhanced Dual Autoencoder Clustering for Liver Histopathological Images".

Our decision is: Revisions Required

=== initial review ===

The word number in abstract should be within 230-250 words and consists of objectives, materials, method, results, and implication for further research.

All the citations should be listed in the references. It is expected that the citations are at least 30 sources, and 70% are the publication within 2020-2026

Please Use Mendeley or other tools for references and citations with IEEE style and include DOI number in each references

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Alde Alanda

Editor
2026-08-19 08:37 AM

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Alde Alanda

Reviewer A:
The manuscript presents a potentially interesting combination of dual autoencoder learning, KAN convolution, ECA, BAM, transfer learning, and clustering for liver histopathology. The topic is relevant and the proposed framework may be useful for unsupervised phenotype discovery. However, substantial methodological and reporting problems currently prevent reliable assessment of the validity and reproducibility of the results.
Clarify the datasets, sample sizes, treatment groups, patch extraction procedure, patient-level splitting, preprocessing, and augmentation strategy. It is essential to exclude leakage between training and testing patches from the same specimen or patient.

Explain precisely how the proposed dual autoencoder differs from a conventional autoencoder and provide complete architectural, optimization, and implementation details.

Resolve inconsistencies in the evaluation. The paper describes an unsupervised task but reports accuracy, precision, recall, and F1-score without defining ground-truth labels or explaining how these metrics were computed.

Report the number of clusters, the procedure used to select it, initialization settings, random seeds, repeated-run statistics, and variability or confidence intervals for all clustering metrics.

Reconsider the interpretation of the clustering results. Internal validity indices and visual inspection alone do not establish that clusters are biologically meaningful or treatment-related.

Add quantitative validation by qualified histopathology experts, comparison with relevant contemporary methods, and preferably evaluation on an independent external dataset.

Correct the apparent inconsistency between the reported Davies-Bouldin value of 3.633 in the main evaluation and the values near 0.4 in Table IV. All tables, figures, and textual claims must be

cross-checked.

Verify the computational-performance measurements. Training time per image, inference time per image, throughput, memory consumption, per-epoch time, and inference time per 1000 patches appear insufficiently defined and potentially inconsistent.

Improve the mathematical notation and presentation. The ELU equation is malformed, the Calinski-Harabasz formula contains a typographical error, and the definitions of the validity indices and variables are incomplete.

Provide stronger ablation experiments. The current ablation table does not clearly isolate all components, does not report statistical variability, and does not demonstrate whether improvements are attributable to the proposed modules or to differences in model capacity.

Moderate claims such as “biologically coherent,” “reliable,” “robust,” and “treatment-related,” unless supported by expert annotation, statistical testing, or independent biological evidence.

Revise the manuscript for language, formatting, figure readability, reference consistency, and journal style. Several passages are repetitive, and some citations appear inserted without sufficient integration into the argument.

Reviewer B:

The most important is the mismatch between the claim of fully unsupervised learning and the use of classification metrics such as accuracy, precision, recall, and F1 without clearly defining how ground-truth labels are used to evaluate clusters. The paper should distinguish internal clustering validation from external label-based validation and introduce appropriate measures such as ARI, NMI, AMI, and Silhouette.

The manuscript identifies a DBI of 3.633 as the main best result, while Table IV reports substantially different DBI values of 0.398 and 0.406. Similarly, the reported inference time of 43.75 ms/image corresponds to approximately 22.9 images/s, whereas 4.9 seconds per 1,000 patches corresponds to approximately 204 images/s. These differences may arise from different experimental settings, but the manuscript does not provide sufficient information to reconcile them.

Since histopathology patches from the same specimen can be highly correlated, the manuscript must demonstrate that splitting was performed at the specimen/slide level before patch extraction. Otherwise, the reported clustering and external classification performance may be overly optimistic.

The biological interpretation is also insufficiently validated. UMAP plots and representative cluster samples cannot by themselves establish that clusters correspond to biologically meaningful treatment-related phenotypes. Expert pathology assessment and/or quantitative morphological measurements are needed to substantiate this claim.

The individual effects of KAN, ECA, BAM, and PANNUKE transfer learning should be evaluated through systematic ablation, while modern deep clustering or self-supervised representation-learning baselines should be considered. Statistical validation across multiple independent runs is also necessary.

Editor
2026-10-14 05:49 PM

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Please send the revision no more than 5 days or will be rejected by the system

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Close